UNDER THE SISTER HOOD

The Unspoken Journey

An Insight Report on Systemic Gaps and Protective Factors in Women's Healthcare

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EXECUTIVE SUMMARY

- Across 45 participants spanning 17 ethnic backgrounds and multiple countries, a clear narrative emerges about the challenges and resilience inherent in women's health journeys.
- Women consistently reframe medical dismissal not as a personal flaw but as a systemic failure, a crucial shift that moves them from self-blame to advocacy.
- This advocacy becomes necessary due to significant gaps in medical education and fragmented care systems, which often fail to address the complex, multifactorial nature of women's health issues from postpartum care to menopause.
- In response to these systemic gaps and the isolation they create, peer storytelling and community connection serve as powerful lifelines, providing the validation and shared experience necessary for collective healing.
- Finally, in the workplace, a pervasive fear of stigma around health disclosures creates a culture of silence, hindering women's career advancement and access to support.

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PARTICIPANT DEMOGRAPHICS

45	24–65	17	30
PARTICIPANTS	AGE RANGE	ETHNIC BACKGROUNDS	REGIONS

AGE DISTRIBUTION

18-24	1
25-34	6
35-44	22
45-54	9
55-64	6
65+	1

Mean: 43.1 | Median: 43

ETHNICITY

Caucasian	19 (42.2%)
WOC	8 (17.8%)
Indian	3 (6.7%)
Mexican	2 (4.4%)
South Africa	1 (2.2%)
Canadian	1 (2.2%)
Japanese	1 (2.2%)
Carribean	1 (2.2%)
Israeli	1 (2.2%)
Afghani	1 (2.2%)
Filipino	1 (2.2%)
Latino	1 (2.2%)
Australian	1 (2.2%)
Trinidadian	1 (2.2%)
Pakistani	1 (2.2%)
Chinese	1 (2.2%)
Nigerian	1 (2.2%)

GEOGRAPHIC SCOPE

33	12
US-Based	International

PA	4
CT	3
CA	3
MA	3
Canada	2
NY	2
AZ	2
NJ	2

+ 22 more

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KEY INSIGHTS

When faced with a healthcare system that dismisses their symptoms or labels them as emotional, women are finding empowerment by reframing the experience not as a personal failing (e.g., being 'crazy' or 'dramatic'), but as evidence of a broken system, which shifts the focus from self-blame to seeking validation and advocating for change.

"Your moods and your hormonal chaos is not a character flaw. It's a biological sign. So first of all, I always say this, you're not crazy, you're not dramatic, and you're not overreacting, right? You are just overdue for supports and overdue to be seen as to be understood."

— PATIENT
EP. UTS_S4_PODCAST_042.DOCX

"it's no need to fix what's not broken because you are not broken. The system is so, healing is more about being seen."

— PATIENT
EP. UTS_S4_PODCAST_042.DOCX

Significant gaps in medical education and training, particularly around hormonal health, menopause, and survivorship care, result in fragmented and inadequate support for women. Patients find their symptoms are often misunderstood or misattributed, such as hormonal issues being treated solely as mental health conditions, forcing them to navigate a system that lacks a holistic understanding of their health.

"I was putting together puzzle pieces, but not just puzzle pieces. It was almost like someone had dumped a box of puzzle pieces. On me. I didn't have the top of what that puzzle looked like, and I didn't know what was survivorship, what was menopause, what was just, you know, uh, mourning the future that I thought that I was gonna have."

— PATIENT
EP. UTS_S4_PODCAST_026.DOCX

"I believe that psychologists and psychiatrists should be educated in hormones because they will be a much better doctors for their patients."

— PATIENT
EP. UTS_S4_PODCAST_028.DOCX

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Complex, multifactorial health issues common to women, such as brain fog or postpartum mental health, are poorly managed by a fragmented healthcare system, forcing patients to navigate siloed specialties and leaving clinicians without clear, integrated treatment protocols.

"It's not like getting a list. Of mental health providers that you now need to call and find out if they're taking your patients, because nobody who's struggling with mental health is going to make that five calls, wait for it, and then go back."

— ADVOCATE
EP. UTS_S4_PODCAST_040.DOCX

"So brain fog, I would say, is one of the hardest things to nail. Like I said, hormone therapy can be helpful, but it often requires us to think a little harder about our nervous system and look a little harder sometimes at what's going on in our life"

— CLINICIAN
EP. UTS_S4_PODCAST_039.DOCX

Patients face significant, systemic barriers to accessing consistent and affordable healthcare, driven by factors like high-deductible plans for low-income individuals, provider shortages due to low reimbursement rates, and insurance plan changes that disrupt established patient-provider relationships. These issues create a fragmented and frustrating care experience, particularly for those managing chronic conditions or life-stage transitions like menopause.

"you'd put somebody in a high deductible healthcare plan. Who's making $40,000 a year who basically can't seek care and didn't have salary to defer. And so some of these things just felt so broken."

— ADVOCATE
EP. UTS_S4_PODCAST_046.DOCX

"if my employer changes a carrier every year and I have to change physicians, it's really hard for me to seek the right type of mental health care...maybe I've just gotten all my menopause on track and now I've gotta switch physicians."

— ADVOCATE
EP. UTS_S4_PODCAST_046.DOCX

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Clinicians, particularly those who are first-generation doctors, experience significant burnout and isolation during their medical training due to a lack of mentorship, financial resources, and formal education on self-care and success beyond clinical skills. This training gap creates a cycle of unwellness where providers struggle to care for themselves, let alone their patients.

"I think by the time I finished medical school, I was already burned out. Like life had been hitting me up inside, you know, I was already burned out. Uh, but no one had, no one told me what success looked like, right? I hadn't seen what success looked like... And if I had a mentor, maybe I would've learned some of those things. But unfortunately, there were no mentors."

— CLINICIAN
EP. UTS_S4_PODCAST_001.DOCX

"'cause you do learn in school. There are some practitioners who are very, you know, strictly psychoanalytic and we're a mirror and we don't show any emotion or whatever, but. You know, I feel like if you're on the ride with me, um, you know, for some people I am a sister, you know, I'm playing a role of a sister."

— CLINICIAN
EP. UTS_S4_PODCAST_003.DOCX

When patients experience dismissal or a lack of answers from the healthcare system, they are compelled to take on the role of researcher and advocate for their own health. This often involves meticulously tracking symptoms, educating themselves on complex medical topics, and learning to persistently question providers and change doctors to overcome diagnostic delays and receive necessary care.

"After when you've been for three years and nobody's helping you. You need to do your own research. You need to advocate and learn and educate yourself."

— PATIENT
EP. UTS_S4_PODCAST_028.DOCX

"so many of the women that I work with, that I know that I interviewed, they have been dismissed by their healthcare providers. Oh, you're just postpartum. What kind of answer is that"

— CLINICIAN
EP. UTS_S4_PODCAST_027.DOCX

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Clinicians find that integrating their personal lived experiences, such as being a parent or managing their own anxiety, into their practice deepens their empathy and builds stronger, more trusting relationships with patients. This contrasts with traditional training that often encourages emotional distance, highlighting that personal vulnerability can be a clinical asset.

"But being a parent and helping parents, it is a whole different experience. It is so nice that I could say to them, you know, I, I, I get it, and I, I'm telling you to do something and sometimes I forget to do it. So it's, it's, you know, I've been there. I know, I understand. It's hard."

— CLINICIAN
EP. UTS_S4_PODCAST_002.DOCX

"The people who are privileged enough and who go into that. Space to sort of help other people often to have the empathy and compassion you have to live through some of it."

— HOST
EP. UTS_S4_PODCAST_001.DOCX

In the face of both individual and collective trauma, actively building and leaning into a peer community is a primary protective factor, fostering a sense of belonging and shared strength that enables collective healing.

"collective trauma leads to collective healing. Which in a way I like that. I like that. I like that a lot. That's good. We look at each other here and we're all in it. And sometimes we forget, but most of the time when you slow down, you realize we're all in it together and we are gonna heal together. We got hurt together, we're gonna heal together."

— ADVOCATE
EP. UTS_S4_PODCAST_016.DOCX

"Lean into your community. Lean into your community. Lean into your community. You know, when this first started the case, many cultures, I went back to the elders, you know, my, my aunts and uncles. What am I supposed to do about this? What's supposed to happen? They've seen these types of things, right? They were in the sixties and they've seen these types of things, and everyone said, community, we rely on each other."

— CLINICIAN
EP. UTS_S4_PODCAST_018.DOCX

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Sharing personal stories and listening to the lived experiences of other women serves as a powerful antidote to isolation, providing the validation, connection, and sense of shared humanity needed to navigate health challenges and systemic failures.

"healing is more about being seen. So what you need is find a place that you know you can feel validated. Connect with other women. You know, listen to other women's stories and don't stay stuck"

— PATIENT
EP. UTS_S4_PODCAST_042.DOCX

"why are we, if, if half of our population is having this, why don't we ever talk about it? Why are we so isolated and why do we struggle in silence?"

— PATIENT
EP. UTS_S4_PODCAST_040.DOCX

Women perceive a significant risk in disclosing health issues—particularly those related to menopause, fertility, or mental health—at work, fearing it could negatively impact their career advancement, leading to a culture of silence that prevents them from seeking necessary accommodations and support.

"I would say there's a majority of employers that are not safe spaces for them to know your healthcare outcomes. And if those leaders know you're seeking, you're currently accessing care for cancer. Maybe you're not up for promotion. If they know you're currently accessing in vitro services, they know you're planning for a baby."

— ADVOCATE
EP. UTS_S4_PODCAST_046.DOCX

"it's no coincidence, right? Because you get to an age where you're ready to advance in leadership and you're confronted with these transitional symptoms that menopause and, and other things can bring."

— ADVOCATE
EP. UTS_S4_PODCAST_045.DOCX

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METHODOLOGY

45

EPISODES ANALYZED

179

MOMENTS SEGMENTED

CONFIDENCE THRESHOLD

Medium and High confidence insights were included in this report.

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